



Pressing need for clinical trial research on dimensional personality disorder

Published Online

November 21, 2024

[https://doi.org/10.1016/S2215-0366\(24\)00365-1](https://doi.org/10.1016/S2215-0366(24)00365-1)

This online publication has been corrected. The corrected version first appeared at [thelancet.com/psychiatry](https://www.thelancet.com/psychiatry) on December 4, 2024

A decade has passed since the introduction of dimensional diagnostic systems for personality disorder.¹ However, national health guidelines (such as those from the UK's National Institute of Clinical Excellence) have not been updated to reflect changes from the previous categorical model of personality disorders.

We conducted a scoping review to examine whether there is enough clinical evidence to warrant the development of national guidelines for dimensional personality disorder. We focused on high-quality evidence from randomised controlled trials (RCTs) on the treatment of dimensional personality disorder, and particularly its recent criterion of severity. We conducted our literature searches from 2013 (introduction of dimensional personality disorder in DSM-5) to Oct 24, 2024, using search terms informed by the latest Cochrane reviews on personality disorder (appendix p 6).

After removing duplicates, we identified 3320 records, of which 862 were considered relevant. Nearly all (99%) relevant reports were based on categorical personality disorders. Most of these (n=517, 60%) focused on borderline personality disorder (BPD), with over half of these (n=320, 62%) being RCTs. The second most researched personality disorder was antisocial personality disorder, with 84 reports, of which 28 (33%) were RCTs. There were only a few RCTs on other personality disorders, including five on avoidant personality disorder, four on narcissistic personality disorder, two on schizotypal personality disorder, and none on the obsessive-compulsive, dependent, schizoid, histrionic, and paranoid personality disorders.

Since 2013, we have identified only three clinical trials on dimensional personality disorder. One of these trials examined a low-intensity intervention for personality disorder, showing promising results in the reduction of social dysfunction but warranting replication due to its pilot nature.² A second study examined the use of personality disorder severity in predicting treatment discontinuation, revealing that among inpatients with personality disorder, those with the lowest levels of self-functioning were the most likely to discontinue therapy.³ A final study conducted secondary longitudinal analyses on a six-year RCT of short-term hospital-based psychotherapy for personality disorder, showing that increases in dimensional personality constructs (eg, relational skills) predicted improvements in global functioning.⁴

The identification of only three trials on dimensional personality disorder points to a concerning lack of RCT-based evidence on the clinical management of this disorder. This absence of RCT research stands in stark contrast to the extensive body of empirical research validating the dimensional conception of personality disorder, including its internal consistency, inter-rater reliability, construct validity, and acceptability by clinicians and patients alike.¹ Recent work also supports the predictive superiority of dimensional personality disorder over categorical personality disorders by revealing that dimensional personality difficulties (such as personality disorder severity and traits) are stronger predictors of various clinical outcomes (including general psychiatric severity, psychosocial functioning, and quality of life), compared to categorical personality diagnoses.¹

Despite these empirical advances, it appears that clinical trial research has not progressed in parallel to investigate how these personality difficulties ameliorate within RCTs. There is a pressing need to conduct

such RCTs, focusing on the treatment of dimensional personality difficulties (including severity, suicidality, and social functioning) as well as whether that treatment is influenced by relevant comorbidities (such as autism and post-traumatic stress).⁵ Only with such rigorous clinical trial research can evidence-based guidelines be created for the management of this serious class of mental health problems.

SP is supported by the University College London Hospitals NIHR Biomedical Research Centre. PF is a Senior Clinical Advisor of the NHS CYP Mental Health Programme. All other authors declare no competing interests.

*Orestis Zavlis, Patrick Luyten, Steve Pilling, Peter Fonagy
orestis.zavlis.23@ucl.ac.uk

Research Department of Clinical, Educational, and Health Psychology, University College London, London WC1E 6BT, UK (OZ, PL, SP, PF); Faculty of Psychology and Educational Sciences, KU Leuven, Leuven, Belgium (PL)

- 1 Sharp C, Miller JD. Head-to-head comparisons of Diagnostic and Statistical Manual of Mental Disorders, fifth edition, section II and section III personality disorder in predicting clinical outcomes. *Pers Disord: Theory Res Treat* 2024; **15**: 275–81.
- 2 Crawford MJ, Thana L, Parker J, et al. Structured psychological support for people with personality disorder: feasibility randomised controlled trial of a low-intensity intervention. *BJPsych Open* 2020; **6**: e25.
- 3 Busmann M, Wrege J, Meyer AH, et al. Alternative model of personality disorders (DSM-5) predicts dropout in inpatient psychotherapy for patients with personality disorder. *Front Psychol* 2019; **10**: 952.
- 4 Klungsoyr O, Antonsen B, Wilberg T. Contours of a causal feedback mechanism between adaptive personality and psychosocial function in patients with personality disorders: a secondary analysis from a randomized clinical trial. *BMC Psychiatry* 2017; **17**: 210.
- 5 Tyrer P, Sharp C. Establishing efficacy and effectiveness in the treatment of personality disorders. *Personal Ment Health* 2023; **17**: 295–99.

Coercive hospitalisation in China

The relationship between individual autonomy in decision making and coercive measures is particularly complex in the field of mental health, especially for people with severe mental illness. In *The Lancet Psychiatry*, Astrid Moell and colleagues¹

See Online for appendix

summarised the complexity and breadth of coercion in hospitalised children and adolescents subjected to coercive measures in mental health services. One coercive measure that deserves more attention in China is compulsory hospitalisation from family requests.

Millions of people with schizophrenia in China² are denied the right to make autonomous decisions because they are perceived as lacking complete self awareness. To reduce the burden on families or for reasons such as stigmatisation, some of these patients have been subjected to forced hospitalisation at the behest of their families for cumulative periods of more than 10 years, which seems to have evolved into a hidden punishment that deprives patients of their freedom. Although long-term compulsory hospitalisation can be used to treat core symptoms in patients with severe mental illness, it neglects other important psychological needs and the basic human rights of patients.

Self-binding directives offer an ingenious way to allow patients to pre-emptively relinquish their autonomy during a crisis.³ However, whether patients can effectively regain their autonomy once their symptoms have improved is often complicated by China's health-care environment and the traditional view that familial, collective, and societal interests take precedence over individual rights. Whether long-term mandatory hospitalisation is truly in the best health interests of patients remains debatable.

In China, we must advocate for increased mental health awareness and support mechanisms within families and push for relevant legislative reforms to safeguard the rights of patients to seek discharge and receive community support. China should promote digital health-care services to help reduce the stress experienced by patients and their families via telemedicine.⁴ China should also continue to explore

the ethical implications of mental health decision making to ensure that patients' voices are heard and respected and to promote more humane mental health practices.

We declare no competing interests.

Yang Zhang, Xingbo Suo, *Jin Gao
gj028960@qlyyqd.com

Department of Clinical Psychology, Qilu Hospital (Qingdao), Cheeloo College of Medicine, Shandong University, Qingdao266000, China (YZ, JG); Department of Psychosomatic Medicine, The 1st Affiliated Hospital, Jiangxi Medical College, Nanchang University, Nanchang, China (XS)

- 1 Moell A, Lyle MS, Rozenal A, Långström N. Rates and risk factors of coercive measure use in inpatient child and adolescent mental health services: a systematic review and narrative synthesis. *Lancet Psychiatry* 2024; **11**: 839–52.
- 2 Huang Y, Wang Y, Wang H, et al. Prevalence of mental disorders in China: a cross-sectional epidemiological study. *Lancet Psychiatry* 2019; **6**: 211–24.
- 3 Stephenson L, Gieselmann A, Gergel T, Owen G, Gather J, Scholten M. Self-binding directives in psychiatric practice: a systematic review of reasons. *Lancet Psychiatry* 2023; **10**: 887–95.
- 4 Galderisi S, Appelbaum PS, Gill N, et al. Ethical challenges in contemporary psychiatry: an overview and an appraisal of possible strategies and research needs. *World Psychiatry* 2024; **23**: 364–86.